

1. Administrative & Study Identification

Full Study Title A Global Study of Volrustomig (MEDI5752) for Participants With Unresected Locally Advanced Head and Neck Squamous Cell Carcinoma Following Definitive Concurrent Chemoradiotherapy **Official Regulatory Title** A Phase III, Randomized, Open-Label, Multi-Center, Global Study of Volrustomig (MEDI5752) as Sequential Therapy Versus Observation in Participants With Unresected Locally Advanced Head and Neck Squamous Cell Carcinoma, Who Have Not Progressed Following Definitive Concurrent Chemoradiotherapy (eVOLVE-HNSCC) **Short Title/Acronym** eVOLVE-HNSCC **Protocol Number** D798EC00001 **NCT Number** NCT06129864 **Sponsor Name** AstraZeneca **Investigational Product** Volrustomig (MEDI5752) / PD-1/CTLA-4 Bispecific Antibody **Phase of Development** Phase III **Medical Monitor** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy (Progression-Free Survival) and safety of sequential therapy with volrustomig compared with observation in participants with unresected locally advanced head and neck squamous cell carcinoma (LA-HNSCC) who have not progressed after receiving definitive concurrent chemoradiotherapy (cCRT). **Secondary Objectives:** To evaluate Progression-Free Survival (PFS) in all randomized participants, Overall Survival (OS) in participants with PD-L1-expressing tumors and all randomized participants, PFS2, safety, patient-reported outcomes, pharmacokinetics, and immunogenicity.

2.2 Background & Rationale Currently, there are no approved maintenance therapies for patients with unresected LA-HNSCC following definitive cCRT. Volrustomig (MEDI5752) is a bispecific monoclonal antibody targeting PD-1 and CTLA-4. This study investigates whether this dual-targeting approach, which amplifies anti-tumor immune responses by enhancing T-cell activation, can prevent disease progression more effectively than the current standard of care (observation).

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator/Observation (Observation arm) **Blinding:** Open-label **Randomization:** 1:1 Randomized

3.2 Planned Enrollment & Duration Target **\$N\$:** 1,145 participants **Number of Sites:** Global trial across ~119 locations (United States + 24 other countries) **Estimated Study Start:** 14-Dec-2023 **Estimated Primary Completion:** 18-Apr-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically or cytologically confirmed LA-HNSCC of the oropharynx, hypopharynx, oral cavity, or larynx. 2. Cancer must be confirmed as unresected Stage III, IVA, or IVB based on the 8th edition of the American Joint Committee on Cancer (AJCC) staging manual. 3. Participants must have completed definitive cCRT with curative intent within 12 weeks prior to randomization and have no evidence of metastatic disease (M0).

4.2 Exclusion Criteria 1. Requiring further treatment with curative intent after definitive cCRT administration. 2. LA-HNSCC that was resected before definitive cCRT or is recurrent at the time of screening. 3. Presence of >1 primary tumor.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Sequential immune checkpoint inhibitor therapy with Volrustomig administered once every 3 weeks (Arm A) vs. Observation (Arm B). **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Maximum of 12 months or 18 cycles, or until RECIST v1.1-defined radiological progressive disease (PD) or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care. **Prohibited:** Further treatment with curative intent after definitive cCRT outside of the protocol guidelines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Within 12 weeks post-cCRT	Informed Consent, Medical History, Tumor Baseline Assessments
Baseline	Day 0	Randomization (1:1), First Dose (Arm A) or Start of Observation (Arm B)
Interim	Every 3 weeks	Checklist review prior to each 3-week cycle dosing, IV Administration (Arm A), Efficacy & Safety Assessments, QoL checks
End of Study	12 Months / 18 Cycles	Final Assessments, Transition to periodic survival follow-up post-progression (remote monitoring)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) in participants with PD-L1-expressing tumors, defined as the time from randomization until first objective radiological progression or death due to any cause. **Measurement Method:** Blinded Independent Central Review (BICR) for tumor assessment per RECIST 1.1 criteria.

7.2 Secondary & Exploratory Endpoints * [Endpoint 1] Overall Survival (OS) in participants with PD-L1 expressing tumors and in

the ITT population. * [Endpoint 2] Progression-Free Survival (PFS) in all randomized participants.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via physical examinations, laboratory tests, and patient reports evaluated according to CTCAE standards. **Serious Adverse Events (SAEs):** Standard reporting timeline, typically within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee configured for global study oversight.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary and secondary efficacy endpoints; Safety Set for all AE/SAE evaluations. **Sample Size Justification:** \$N=1,145\$ participants sized to adequately power the detection of statistically significant improvements in PFS and OS. **Handling of Missing Data:** Standard survival analysis censoring rules applied to time-to-event data (e.g., PFS and OS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits for data verification globally. **Audit Trail:** Strict documentation of eCRF locking, source data verification, and query resolutions. **Education Protocol:** HCPs will receive site initiation and protocol adherence training. Patients will receive education regarding informed consent and adverse event reporting guidelines.

11. Study Identifiers & Governance

Primary ID: {{D798EC00001}} **Registry Number:** {{NCT06129864}} **Sponsor/Lead Organization:** {{AstraZeneca}} **Study Title:** {{eVOLVE-HNSCC}} **Official Regulatory Title:** {{A Phase III, Randomized, Open-Label, Multi-Center, Global Study of Volrustomig (MEDI5752) as Sequential Therapy Versus Observation in Participants With Unresected Locally Advanced Head and Neck Squamous Cell Carcinoma, Who Have Not Progressed Following Definitive Concurrent Chemoradiotherapy (eVOLVE-HNSCC)}}

12. Clinical Framework (Oncology Specific)

Primary Condition: {{Locally Advanced Head and Neck Squamous Cell Carcinoma (LA-HNSCC)}} **Diagnosis Threshold:** {{Unresected Stage III, IVA, or IVB without progression post-cCRT}} **Medical Methodology:** {{Sequential immune checkpoint inhibitor therapy}}

Optimization Target: {{PD-1/CTLA-4 dual inhibition to prevent progression}}

13. Study Procedures & Methodology

Management Pathway: {{Sequential Volrustomig IV vs. Observation}}
Education Protocol (HCP): {{Site initiation and protocol adherence training}} **Education Protocol (Patient):** {{Informed consent and adverse event reporting guidelines}} **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) for tumor assessment}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 12 months or 18 cycles of therapy, plus survival follow-up}} **Onsite Visit Cadence:** {{Every 3 weeks (Q3W)}} **Remote Monitoring Frequency:** {{Periodic survival follow-up post-progression}} **Checklist Review Interval:** {{Prior to each 3-week cycle dosing}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---		:---		:---		:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]									
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]									
Statistician	[Name]	_____	[DD-MMM-YYYY]									